FOLFIRI

Day 1: irinotecan, folinic acid and fluorouracil, then a 46-hour fluorouracil pump. A cycle lasts 14 days; your team confirms the next treatment.

Draft for clinical, pharmacy and patient/PPI review. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hair

Irinotecan

Hair thinning or loss may occur. Ask about support.

2 Mouth

Fluorouracil + folinic acid

Report ulcers or pain affecting eating and drinking.

3 Chest

Fluorouracil

New chest pain or breathlessness needs urgent assessment.

4 Bowel

Irinotecan / fluorouracil

Report diarrhoea, increased stoma output or severe abdominal pain.

5 Hands / feet

Fluorouracil

Report painful redness, peeling or loss of hand function.

6 Blood / infection

Chemotherapy

Fever, shivers or sudden illness: call oncology immediately.

7 Skin / hands / feet

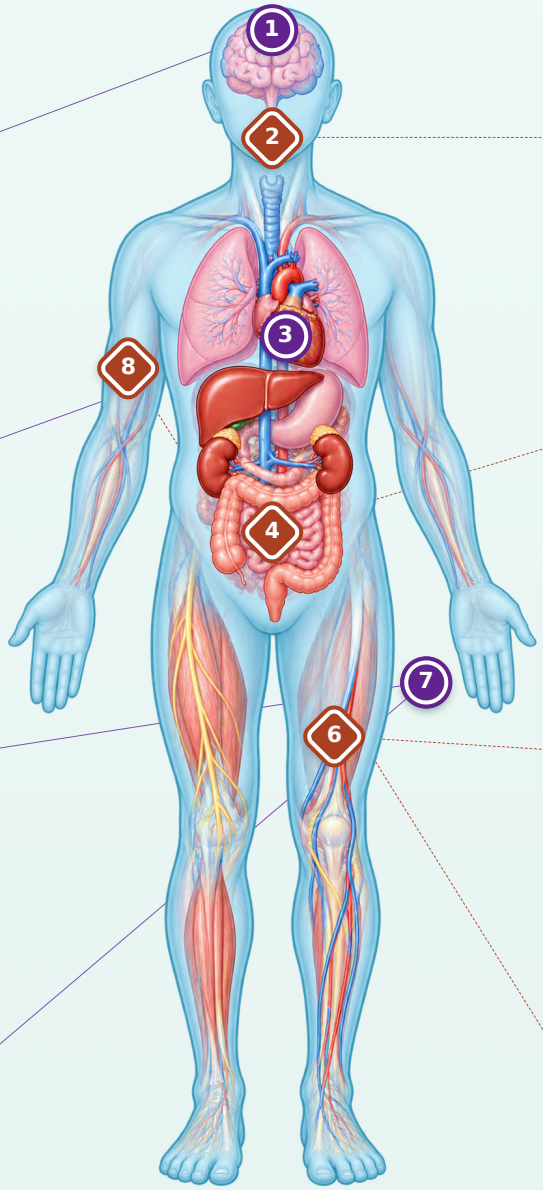
Fluorouracil

Report painful skin changes affecting activity.

8 Pump / line

Treatment delivery

Pain, leakage, swelling or poor wound healing needs advice.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

NCCP 00227 · PRACTICAL SUPPORT

Your treatment, at home.

Important medicine-specific effects

Irinotecan

- Early cramps, sweating and diarrhoea
- Delayed diarrhoea and dehydration
- Low blood counts / serious infection
- Nausea, mouth soreness and fatigue
- Hair thinning or loss
- Rare serious lung or allergic reactions

Folinic acid

- Enhances fluorouracil activity and toxicity
- Rare allergic reaction
- Diarrhoea / mouth soreness in combination

Fluorouracil

- Diarrhoea, dehydration and mouth ulcers
- Low blood counts / infection
- Hand-foot skin soreness
- Cardiotoxicity including chest pain / ischaemia
- Severe early toxicity including DPD-related toxicity
- Pump / central-line complications

Make the plan practical

DPD testing before fluorouracil

DPD helps the body break down fluorouracil. Deficiency can cause severe toxicity. Testing guides the plan but cannot exclude all serious reactions; unexpectedly severe early diarrhoea, mouth sores or illness needs immediate review.

Your diarrhoea plan

Have the prescribed anti-diarrhoeal medicine and written instructions before going home. Follow that plan as soon as diarrhoea starts. Call promptly for persistent symptoms, cramps, fever, blood in stools or reduced urine.

Pump and line care

See “Your port and treatment pump” for insertion, connection, daily care and disconnection. Before leaving, know who to call, where to return and how to use the supplied spill plan.

Anti-sickness support

Take prescribed preventive medicines and keep the breakthrough plan available. Some anti-sickness medicines cause constipation. Ask for review if symptoms are poorly controlled.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital’s instructions. If you cannot reach the team, seek urgent medical assessment.

CUH · MON–FRI 8AM–5PM

087 181 9645

Acute Oncology Service

CUH · WEEKENDS / BANK HOLIDAYS 9AM–5PM

021 4922000

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00227/

Sources and review status are available online. Follow your own prescription and hospital instructions.